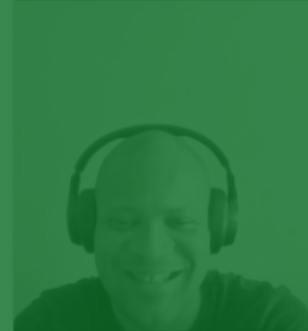
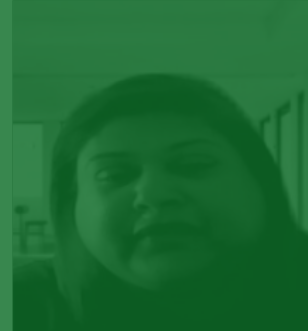
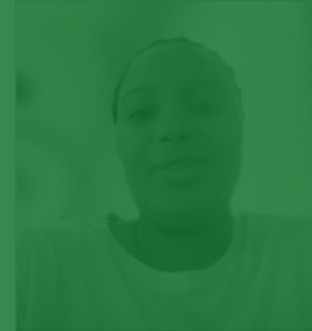
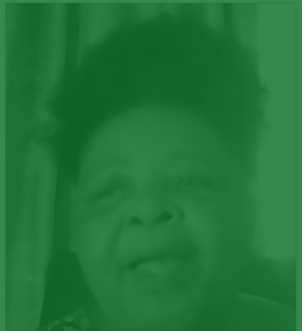
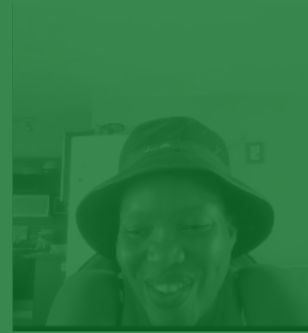
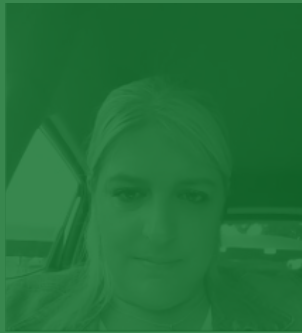




Tribe 4: Enterprise Acceleration
***Medication collection &
delivery journey***

In-depth interviews



Questions we wanted to answer

Main research question

How do Dis-Chem customers experience getting pre-packed prescription medication, either through collecting in-store or having it delivered to their home or workplace?

Sub-questions

- **Choice & motivation:** *Why do customers choose in-store collection or delivery for pre-packed prescriptions?*
- **Journey performance:** *What works well, what causes friction, and where do customers rely on workarounds across the redemption journey?*
- **Differences & constraints:** *How do experiences vary by channel (collection vs delivery), medication type (chronic vs acute), and responsibility (self vs managing for others)?*
- **Impact & priorities:** *Which changes would most improve clarity, reliability, and ease in the redemption experience?*



Research focus

Goal:

To understand how customers experience the redemption of pre-packed prescription medication at Dis-Chem, across both in-store collection and home/workplace delivery, and to identify opportunities to improve clarity, reliability, and ease at key moments in the journey.

Scope:

Script submission → notification → planning → arrival / waiting → handover → counselling → payment / confirmation → exit.

Why this matters

Previous research has consistently highlighted friction points during prescription redemption, even where pre-packed services are intended to save time and reduce effort.

This study builds on that foundation to:

- *Understand where friction and moments of delight persists in real customer experiences.*
- *Identify the moments that matter most during collection and delivery.*
- *Reveal where customers rely on manual follow-ups and workarounds.*
- *Separate experience, communication, and operational drivers of friction.*





What's in this report

Planning

- *Research focus*
- *Research questions*

Participants

- *Participant demographics*

Key insights

- *Household & health-management roles*
- *Fulfilment motivations & frictions*
- *Digital behaviour, app usage & loyalty*
- *Mental models: how customers think about “getting medication”*

Script fulfilment journeys (insights and recommendations)

Summary and next steps

About the research



PRE-ORDERED
PRESCRIPTION
COLLECTIONS ONLY

PLEASE QUEUE
AT DISPENSARY

To do this, we interviewed
15 Dis-Chem customers on
their medication collection and
delivery journeys

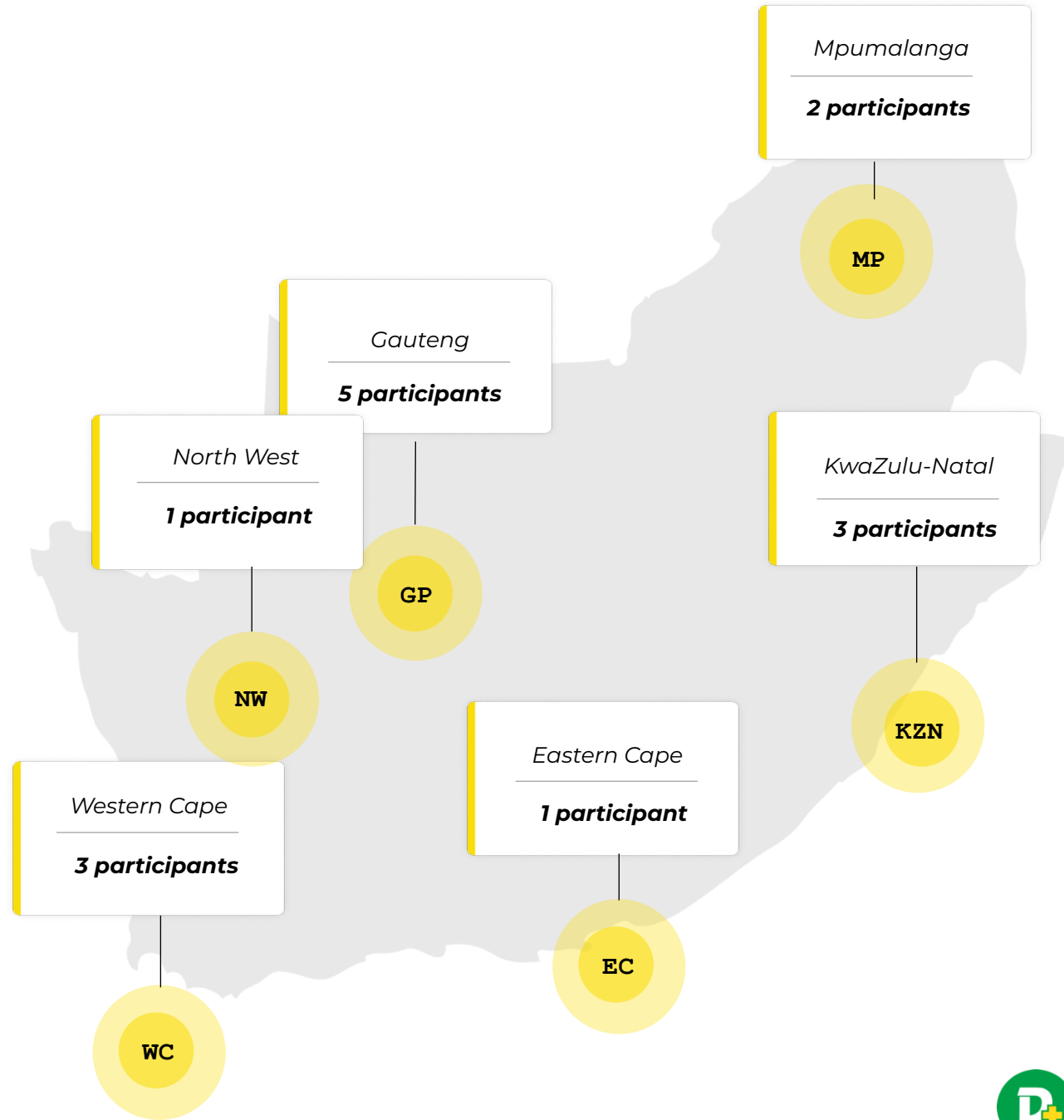


15 in-depth interviews

We facilitated 1-hour remote one-on-one sessions.

Recruitment criteria

- **15 Dis-Chem customers** who have redeemed pre-packed prescription medication at least **twice** in the last **120 days**:
 - 8 who collected pre-packed medication in-store
 - 7 who received pre-packed medication via delivery
- Age range: 25–60 years
- Balanced, representative mix across **gender, race, and provinces** (urban, suburban and rural areas across South Africa).





PARTICIPANT DEMOGRAPHICS

Gender

15 participants

The participant group skews slightly female, reflecting the reality that women more often take responsibility for managing household health and medication errands.

Female 9/15



60%

Male 6/15



40%





PARTICIPANT DEMOGRAPHICS

Age

15 participants

25-34 4/15



27%

35-44 3/15



20%

45-54 6/15



40%

55+ 2/15



13%

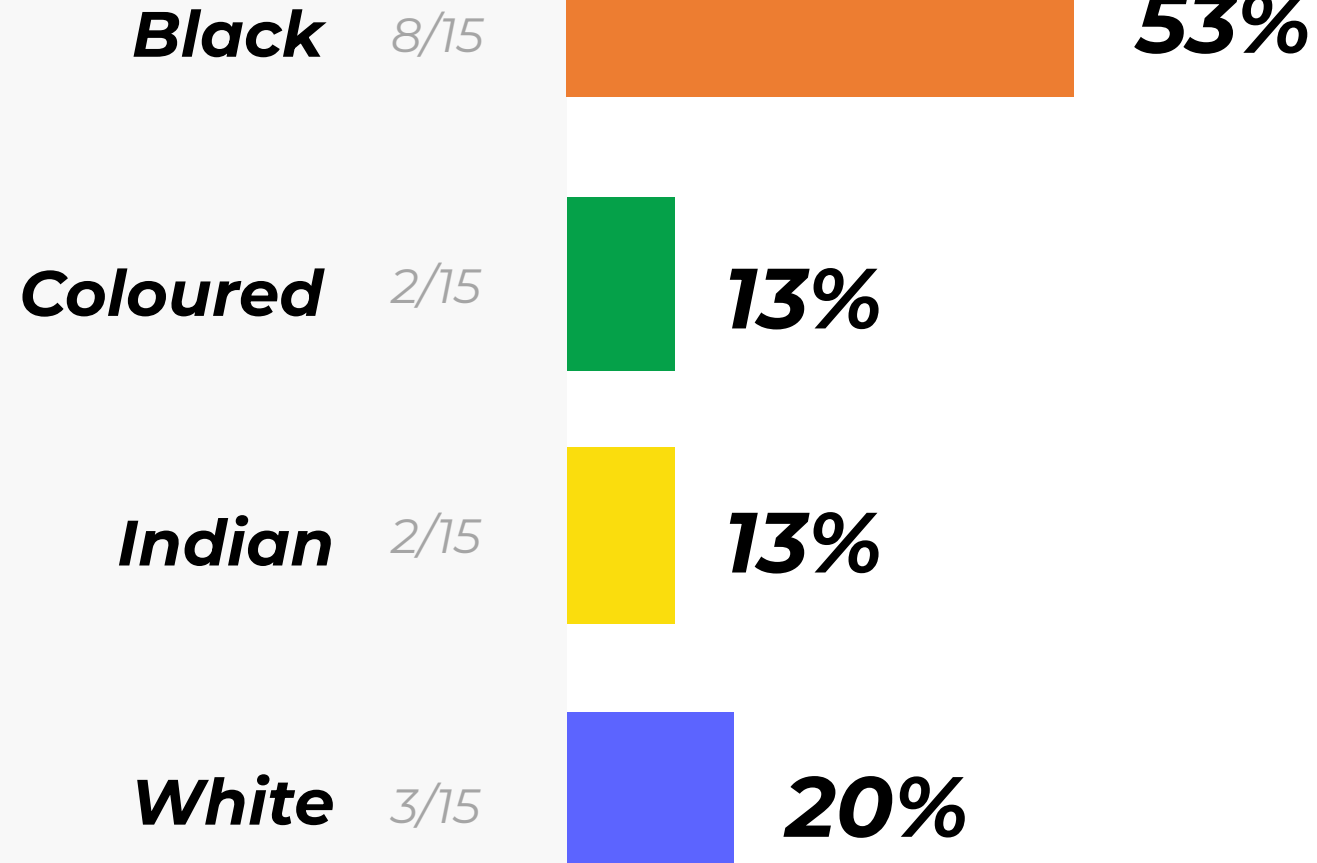


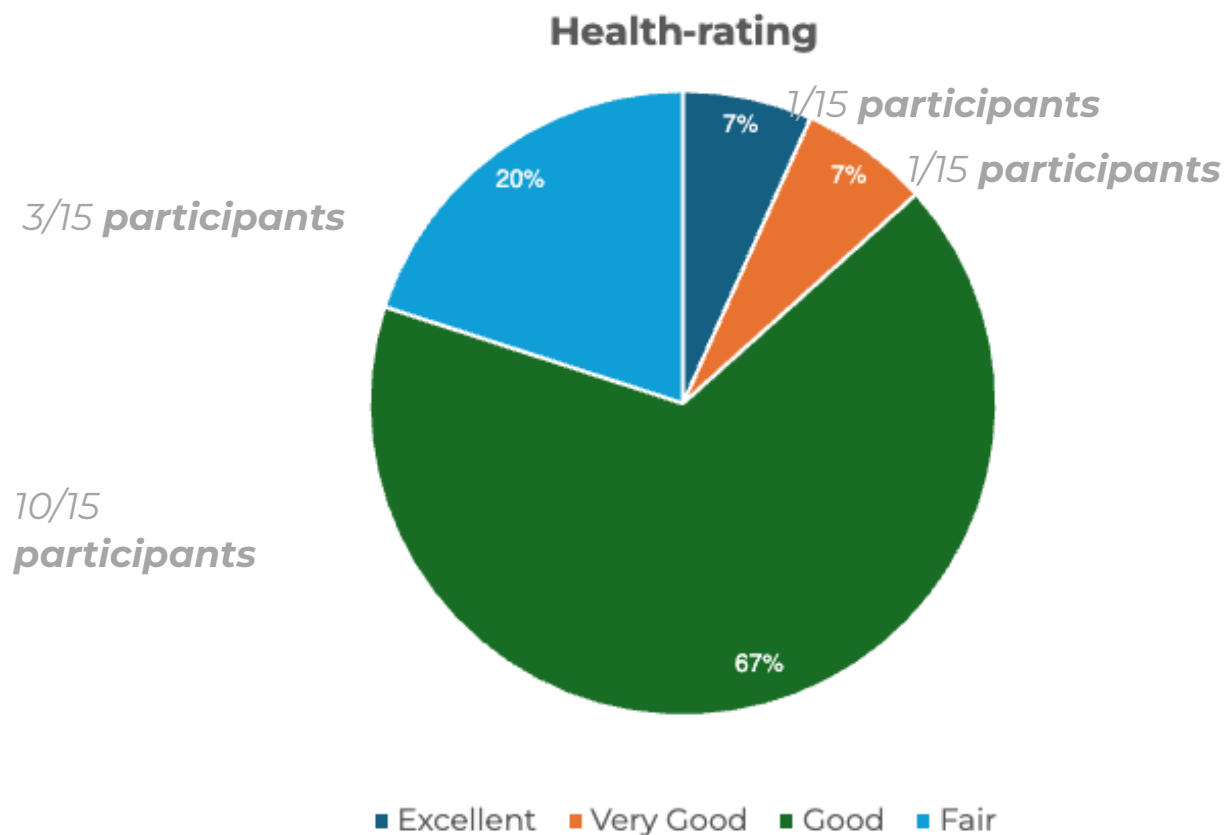


PARTICIPANT DEMOGRAPHICS

Race

15 participants





Health profile and management context

Across both cohorts (collection and delivery), health is framed less as something to optimise and more as something to keep **stable** and **manageable**.

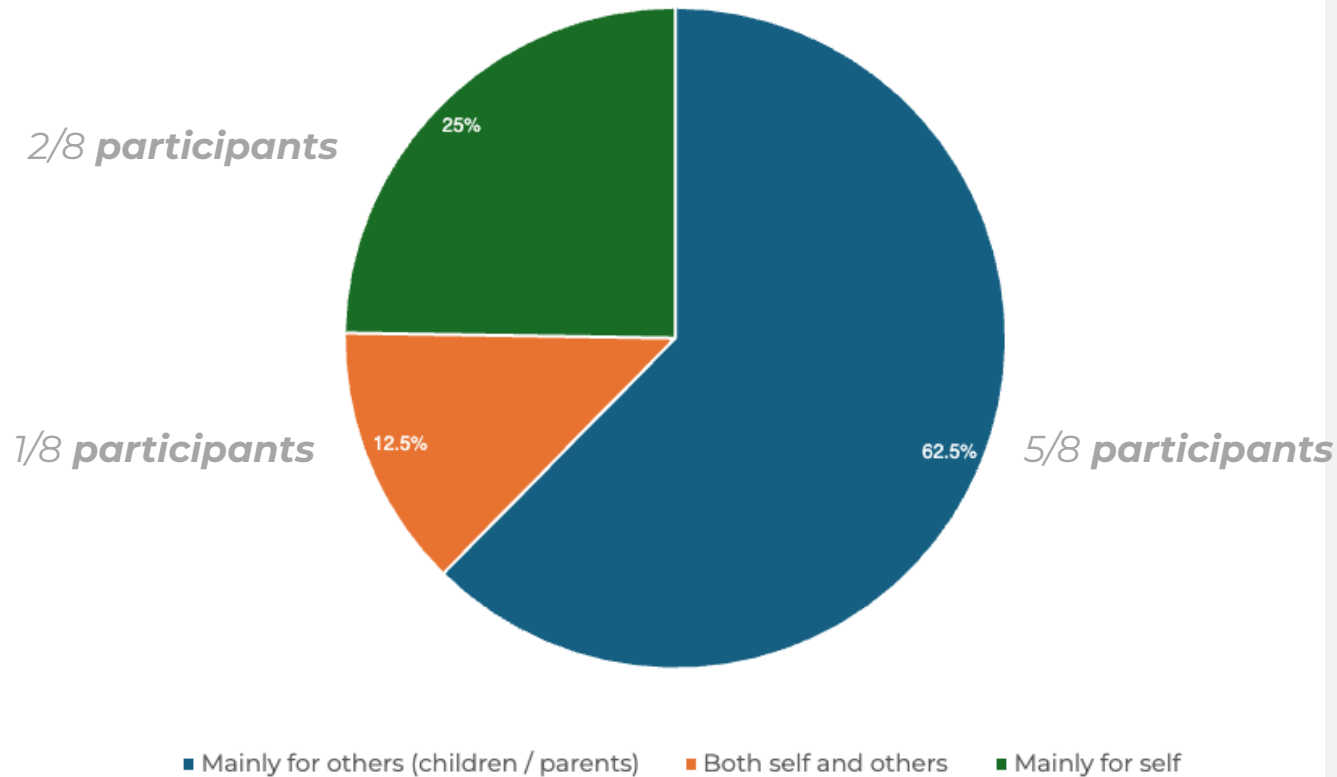
Most participants rate their health as **good** or very **good**, even when managing chronic conditions. This reflects a mindset where medication is seen as a **practical tool** to maintain normal daily functioning:

- The priority is **not missing doses**, avoiding disruption, and keeping existing routines running smoothly.
- For chronic users, medication adherence is described as **non-negotiable** and, in some cases, life-critical.
- For those managing medication on behalf of others, health tasks are embedded into **broader household responsibilities** rather than treated as standalone events.



PARTICIPANT DEMOGRAPHICS: COLLECTION

Who the collection service is used for



Medication use: in-store collection

- Participants using in-store collection are most often managing medication on **behalf of others**, including children, elderly parents, or multiple dependents within the same household.
- Medication needs in this cohort are **mixed and layered** combining chronic repeats, acute prescriptions, and, in some cases, controlled medication that requires in-person pickup. This creates a higher coordination load and more frequent interaction with the pharmacy.
- As a result, collection is not just about convenience, but about maintaining control **across multiple prescriptions, timelines, and people.**



Medication use: delivery

- **4 of 7 participants** manage medication mainly for themselves
- **3 of 7 participants** manage medication on behalf of others (e.g. family members)

Delivery users are typically managing their **own chronic or repeat medication**, where the primary need is reliability rather than flexibility.

Unlike in-store collection, delivery is not used to juggle multiple prescriptions, people, or timelines, it is used to **lock in a predictable monthly routine** with minimal cognitive and logistical effort.

Delivery becomes part of a **background system** that “just works,” allowing participants to focus on work and household responsibilities without repeated pharmacy touchpoints.

What this means in practice

- Delivery is optimised for continuity and low-effort maintenance, rather than control across complexity.
- Participants value delivery when medication needs are stable, repeatable, and non-urgent.
- Compared to collection, delivery reduces the need for active monitoring, timing decisions, and in-store interaction.



Key insights:

1. health management roles
2. Fulfilment motivations & frictions
3. Digital behaviour, app usage & loyalty
4. Mental models: how customers think about “getting medication”

1. One person usually carries responsibility for household medication

- Across both collection and delivery cohorts, a single household member typically manages prescriptions, repeats, and fulfilment decisions for others.
- This role often includes children, partners, or elderly parents, even when the participant is not chronically ill themselves.
- Medication fulfilment is therefore less about “my prescription” and more about **keeping the household functioning**.

2. Health management is embedded into everyday life, not treated as a separate task

- Participants describe fitting medication errands around work, school runs, caregiving, and shopping.
- Fulfilment methods are chosen to **minimise disruption** rather than **optimise for novelty or digital features**.
- Reliability matters because medication tasks sit on top of already busy lives.

What participants said

“

I'm still the mom of everybody... they all look to me to get it done.”

- Collection participant

“

You're trying to balance work, health, and family — you can't afford for things to go wrong.”

- Delivery participant

3. Managing medication means managing coordination, not just access

- Many participants **juggle multiple prescriptions, timelines, and rules** (monthly repeats, six-month scripts, controlled meds).
- This creates a higher **cognitive load** and increases sensitivity to breakdowns in communication or timing.
- As a result, predictability and control are valued more than speed alone.

4. Medication fulfilment carries personal risk, not just effort

- Across both collection and delivery cohorts, participants implicitly **take responsibility for the consequences of failure**.
- Missed doses, delayed treatment, or stress for dependents. As a result, **predictability and reliability consistently matter more** than speed or convenience.
- This helps explain why participants rely on routines, familiar branches, and manual follow-ups: they act as safeguards against uncertainty in a system they don't fully control.

What participants said

“

If you go in too early they say no, you're not due yet... and if you're late, you've missed your medication.”

- Collection participant (managing controlled medication)

“

I can't skip that... my life is at risk.”

- Collection participant (chronic medication)

How customers choose collection vs delivery

1. Fulfilment choices are about fitting medication into daily life

- Across **both cohorts**, customers choose fulfilment methods that minimise disruption to work, family, and caregiving responsibilities.
- The decision is **practical** rather than ideological as customers are not choosing a “channel,” but the option that best fits into already full schedules.

2. Collection is chosen for control, visibility, and certainty

- Collection users value being able to physically hand in scripts, choose when to collect, and receive medication directly from a familiar branch.
- This sense of **visibility** and **control** is especially important when managing medication for others or dealing with controlled or high-risk prescriptions.

What participants said

“

I need it to fit around my work, not the other way around.”

- Delivery participant

“

Once I send it (script), I assume they'll sort it out but I still want to know it's there.”

- Collection participant

3. Delivery is chosen to remove repeated effort

- Delivery users consistently describe delivery as “easier” because it **eliminates recurring tasks** such as travelling to the pharmacy, standing in queues, and planning repeat visits.
- Once set up, delivery becomes **part of a background routine** rather than an ongoing decision.

4. Urgency and medication type override channel preference

- Regardless of cohort, participants share a clear rule: urgent or once-off medication requires in-store collection, while chronic or repeat medication is suited to pre-packed collection or delivery.
- Fulfilment choices flex based on **risk, urgency, and regulatory** requirements.

5. Reliability matters more than speed or digital sophistication

- Across both cohorts, **trust and predictability outweigh speed or digital features**.
- Customers are willing to use manual follow-ups, familiar branches, or routines if it reduces uncertainty around whether medication will be ready or arrive on time.

What participants said

“

It's just easier to get it delivered.”

- Delivery participant

“

If it's urgent, I'll go in. Delivery is for the usual stuff.”

- Delivery participant

1. Digital comfort does not equal digital preference for medication

- Participants are digitally confident in everyday life, but **do not treat medication like other digital services**.
- Even regular users of apps and online tools prefer familiar, low-risk channels for medication.

2. Email and WhatsApp are used for ease and reassurance

- **Email and WhatsApp** are chosen because they are simple, familiar, and don't require learning something new.
- They also provide **visible proof** that a script was sent, which matters when medication is important.

3. The Dis-Chem app plays a limited, supporting role

- Most participants either don't use the app actively or use it only to receive notifications.
- The app does not replace WhatsApp, email, phone calls, or in-store confirmation for critical steps.

What participants said

“

This is medication — it's not the same as ordering food.”

- Delivery participant

“

I just WhatsApp it because it's quick.”

- Collection participant

“

I have the app, but I don't really use it.”

- Collection participant

4. Loyalty is driven by familiarity, not digital engagement

- Across both cohorts, loyalty to Dis-Chem is **long-standing and habitual**, built over time through familiarity with specific branches, staff, and processes rather than through digital features or rewards optimisation.
- Participants rarely describe choosing Dis-Chem because of loyalty benefits; instead, loyalty reflects **trust** in the pharmacy to reliably handle medication, especially for chronic or family prescriptions.
- Engagement with loyalty mechanics is **light and passive** i.e. cards are forgotten, points are used when convenient, and newer programmes are sometimes unknown without affecting continued use of Dis-Chem for medication.

What participants said

“

I've always used Dis-Chem.”

- Collection participant

“

I didn't even know about the new rewards.”

- Collection participant



How customers think about medication fulfilment

Understanding the mental models behind collection & delivery behaviour

Why this matters

Across this study, customers consistently prioritised reliability, predictability, and control often at the expense of speed, automation, or “convenience” as traditionally defined.

At first glance, this can look like resistance to digital channels or newer fulfilment options. In reality, these behaviours reflect **deeply held mental models about medication and risk**.

What we mean by mental models

Mental models are the underlying beliefs people hold about:

- What could go wrong
- What matters most
- Where responsibility sits
- How much trust they place in systems versus themselves

These models shape behaviour more strongly than features, channels, or interfaces.

- Customers do not experience prescription fulfilment as a retail or e-commerce journey.
- They experience it as a **high-stakes health responsibility** that must fit into already complex lives.



To improve the experience, we need to design **for how customers think** about medication not how we **expect them** to behave.

Mental model 1: medication is treated as **a high-risk responsibility**

Medication errors have real consequences

Delays, errors, or missed doses affect health, dependents, and daily functioning.



This creates a strong need for certainty and control

Customers want to know where their medication is, who has it, and when it will arrive.



Customers default to familiar, proven routines

Known branches, repeat cycles, WhatsApp/email, call-centre reminders, manual follow-ups.



Fulfilment choices optimise for predictability over innovation

Collection is chosen for visibility and control; delivery is chosen for stable, repeat needs.

Customers are not resisting digital or new services, they are protecting themselves from risk.

*Any change to prescription fulfilment must **increase certainty, visibility, and trust** before it can feel like an improvement.*

Mental model 2: medication is not treated like regular online shopping

Getting medication is fundamentally different from buying everyday goods

Medication is tied to health outcomes, dependents, and risk — not convenience, speed, or choice.

Customers do not browse, compare, or experiment

Customers are not “shopping around” when fulfilling prescriptions. They want the same outcome every time, with minimal decision-making.

Control and reassurance matter more than flexibility

Customers value: Knowing who has their medication, knowing where it is and knowing when it will be ready or arrive.

Digital success is defined by reassurance, not features

Medication tracking, confirmations, and reminders matter more than: speed, personalisation, discovery or promotions.

Customers are not looking for a “better shopping experience.” They are looking for **confidence that nothing will go wrong.**

Designing medication fulfilment like retail risks:

- Increasing cognitive load
- Introducing uncertainty
- Undermining trust

Digital tools must **reassure first**, optimise second.

Mental model 3: responsibility changes behaviour

Medication fulfilment is often done on behalf of others

Many participants manage medication for children, elderly parents and partners or dependents.



Responsibility amplifies risk sensitivity

When someone else's health is involved: delays feel more stressful, errors feel less tolerable and follow-ups feel necessary.



People build safeguards into the journey

Customers compensate for uncertainty by choosing familiar branches, using known staff or call-centre contacts, following up manually or avoiding unfamiliar or opaque processes



Predictability is valued over speed or convenience

Customers would rather: wait longer, travel further and use "older" channels. If it means greater confidence in the outcome.

Responsibility fundamentally changes how customers behave. Systems that assume:

- Individual-only users
- Low-risk transactions
- Passive trust

Will fail people who are **actively protecting others**.

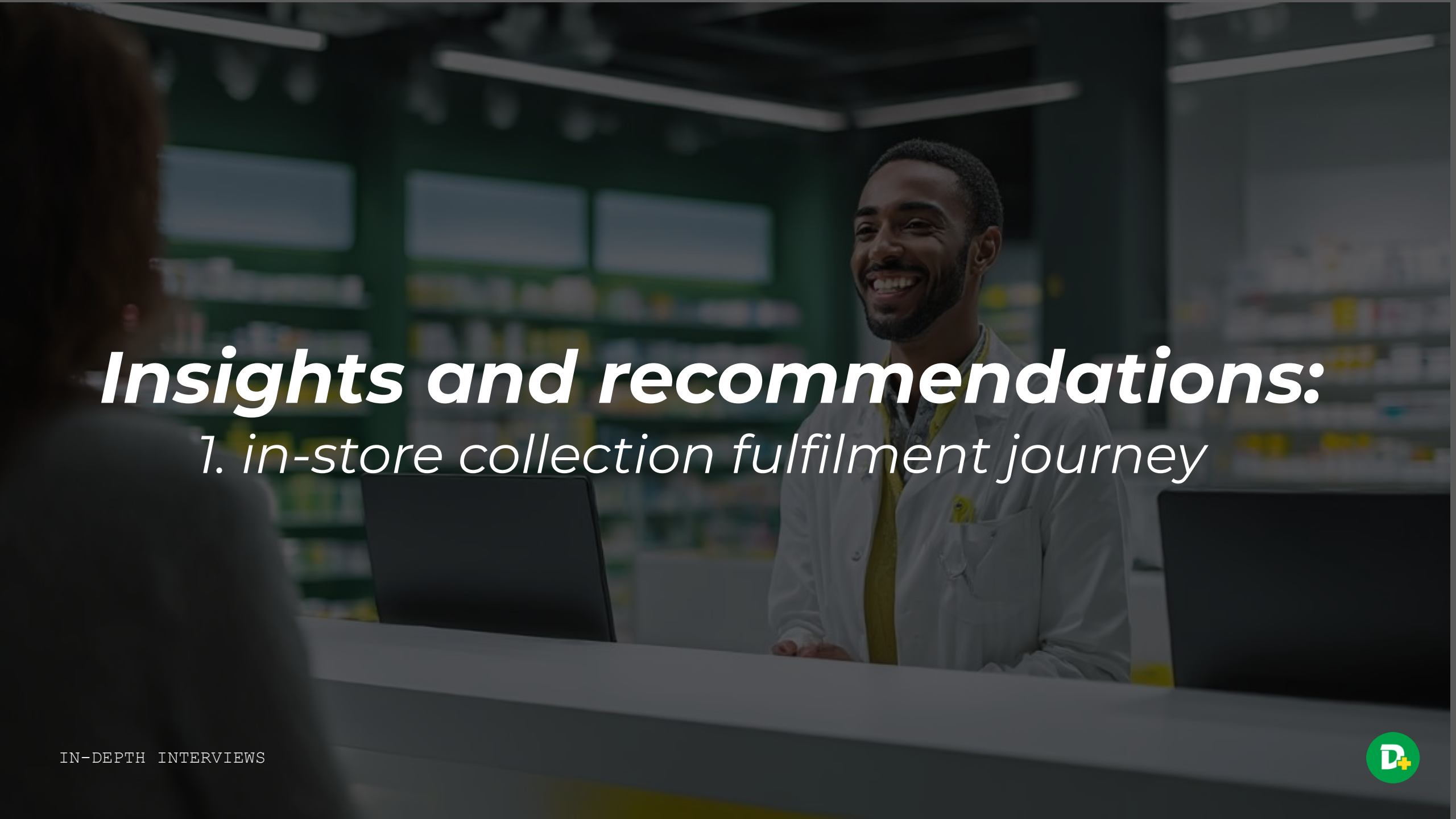
Design must support customers as care coordinators, not just end users.



Script fulfilment journeys: *insights and recommendations*

IN-DEPTH INTERVIEWS





Insights and recommendations:

1. in-store collection fulfilment journey

SCRIPT JOURNEY OVERVIEW: COLLECTION

Stage 1: script submission / intake

Covers how customers receive prescriptions and initiate **pre-packed collection** at Dis-Chem. The focus is on ensuring the script is **safely handed over** to a familiar branch so medication can be prepared without ongoing follow-up

Stage 2: notification & deciding when to collect

Covers how customers are **notified** that medication is ready and **how** they decide when to go and collect it. Decisions are driven less by real-time notifications and more by **routine, timing buffers, and everyday schedules** such as work, school, and shopping.

Stage 3: arriving at the store, finding the counter & waiting

This stage covers what happens when customers **arrive** at the Dis-Chem store to collect their medication, including how they **navigate** to the pharmacy and **experience waiting**.

Stage 4: at the collection counter

Covers the **interaction** at the **pharmacy collection counter**, where customer details are verified and pre-packed medication is handed over.

Stage 5: counselling / information

Covers how customers **receive information** about how to use their medication at the point of collection, and how much they **rely** on that interaction.

Stage 6: payment & leaving the store

Covers **payment** and **exit** after medication collection. This is often where the biggest remaining friction occurs, as pharmacy efficiency gives way to general store checkout processes.



Stage #1:

Script submission /intake



Behavioural patterns

- **Physical scripts remain central to trust:** Many still receive handwritten or printed scripts particularly for renewals and controlled medication. Dropping the script off in person provides reassurance that it has been received and logged correctly.
- **Digital submission shifts effort, not responsibility:** Email and WhatsApp are used pragmatically to send scripts ahead but customers still feel responsible for ensuring the pharmacy has received and processed them.
- **Multiple medication types create parallel processes:** Households often manage several pathways at once (chronic, controlled, acute), each with different rules and timelines, increasing coordination effort.
- **Branch familiarity anchors the handover:** Participants usually submit scripts to one known Dis-Chem branch. Familiar staff and routines reduce anxiety about loss, delays, or errors.

What participants said

"I normally just drop it off there so I know they have it."

— Collection participant

"I sent it on WhatsApp, but I still wait to see if they've actually loaded it."

— Collection Participant

"I always take it to the same Dis-Chem, they know me there."

— Collection Participant



What worked well

- ✓ **Clear responsibility handoff:** Once a script is submitted, participants expect the pharmacy to take over preparation without further action.
- ✓ **Stable repeat routines:** Where repeats are well set up, medication is prepared predictably with minimal effort from customers.
- ✓ **Direct doctor-pharmacy submission (when available):** When doctors send scripts directly to Dis-Chem, the process feels simpler and more reliable.

What caused friction

- ✗ **Lack of confirmation creates uncertainty:** Uncertainty whether emailed or WhatsApped scripts were received, leading to follow-ups and stress.
- ✗ **Script loss and duplication:** In some cases, scripts need to be resent by doctors, with patients acting as proof-holders between systems.
- ✗ **No single confirmation loop:** Multiple actors (doctor, PA, pharmacy, call centre, patient) operate without a shared confirmation mechanism.

Next steps & recommendations



Standardise digital script submission across branches:

Centralise and formalise email and Business WhatsApp handling so scripts are processed consistently, with clear ownership and response standards regardless of branch.



Create a clear confirmation moment after script handoff:

Provide explicit confirmation when a script has been successfully received and loaded (not just sent), for both physical and digital submissions.



Align confirmations and reminders to medication cycles:

Trigger confirmations and reminders based on real-world patterns, such as monthly repeats, six-month renewals, and controlled medication rules, to reduce missed doses and follow-ups.

Stage #2:

*Notification & deciding when to
collect*



Behavioural patterns

- **Notifications confirm readiness, not timing:** SMS messages are received inconsistently and are often used simply to confirm that medication is ready, rather than to plan an exact collection time.
- **Routine outweighs communication:** Many collect based on established habits (monthly cycles, known due dates) and will go to the store even without explicit confirmation.
- **Direct contact fills gaps in uncertainty:** When clarity is needed, some customers bypass automated messages and phone the pharmacy or rely on WhatsApp confirmation from a known contact.
- **Collection timing is shaped by life logistics:** Work schedules, school runs, shopping trips, and buffer planning (collecting a few days early) play a larger role than notifications themselves.

What participants said

“I don’t always wait for the SMS — I just go.”

— Collection participant

“I usually collect a few days before so I don’t run out.”

— Collection participant



What worked well

- ✓ **Notifications provide reassurance rather than instruction:** Even when timing details are vague, receiving an SMS or message reassures customers that their medication is in progress and not forgotten.
- ✓ **Established routines reduce dependency on messaging:** Many customers rely on monthly habits, known due dates, or repeat cycles, which allows them to plan collection without waiting for confirmation every time.
- ✓ **Direct contact offers clarity when needed:**
The ability to phone the pharmacy or receive a WhatsApp confirmation provides confidence when notifications are unclear or missed.

What caused friction

- ✗ **Notifications are inconsistent and easy to overlook:** SMS messages are sometimes missed, delayed, or ignored, especially when customers are busy or receive many automated messages daily.
- ✗ **Unclear status creates hesitation and extra effort:** Customers are often unsure whether a script has been received, packed, or is actually ready, leading to unnecessary trips or follow-up calls.
- ✗ **Customers compensate for gaps with manual follow-ups:** In the absence of clear, reliable updates, customers take responsibility for checking status themselves by calling, messaging, or arriving in-store “just in case.”



Next steps & recommendations



Clarify readiness at every step: Clearly distinguish between script received, medication packed, and ready for collection across SMS, app, and in-store touchpoints.



Use QR-based collection to reinforce readiness and speed: As QR scanning for collection is rolled out, use it as a clear signal that medication is ready and verified reducing uncertainty at arrival and reinforcing the “fast lane” expectation for pre-packed collection.



Align notifications with real collection routines: Trigger messages around monthly repeats, refill dates, and controlled medication schedules, rather than one-off alerts.



Reduce manual follow-ups through clearer messaging: Include explicit next-step guidance so customers don't need to phone or WhatsApp to confirm status.



Stage #3:

*Arriving at the store, finding the
counter & waiting*



Behavioural patterns

- **Familiarity drives confidence and speed:** Most participants go straight to the pharmacy or collection point because they know the store layout and process well.
- **Dedicated collection points signal efficiency:** Separate collection counters, bells, or shorter queues reinforce the expectation that pre-packed collection should be faster than standard dispensing.
- **Waiting is acceptable until context changes:** Short waits are tolerated, but delays feel significantly more stressful when customers are collecting for others, especially sick children.
- **Errand-bundling affects arrival behaviour:** Some shop for toiletries or other items before going to the pharmacy, while others go directly to collect medication first.

What participants said

“I just walk straight there — I know exactly where to go.”

— Collection participant

“The collection counter is quick because not many people use it.”

— Collection participant



What worked well

- ✓ **Familiar layouts reduce cognitive effort:** Knowing where to go removes uncertainty and speeds up the visit.
- ✓ **Separate collection counters preserve the ‘fast lane’ promise:** Dedicated points reinforce that pre-packed collection is quicker and more predictable.
- ✓ **Predictable queues enable errand-bundling:** Customers feel comfortable combining collection with other shopping when wait times are short or known.

Frictions

- ✗ **Delays undermine the value of pre-packing:** System issues or staff availability can erase the perceived time-saving benefit.
- ✗ **Stress spikes when collecting for others:** Waiting with children or for essential medication heightens frustration.
- ✗ **Payment queues break momentum:** Fast collection followed by slow checkout creates a fragmented experience.



Next steps & recommendations



Make collection points more visible and consistent: Ensure dedicated collection counters or bells are clearly signposted and consistently available across branches.



Set expectations for wait times: Communicate when longer waits may occur (e.g. peak periods or new scripts). This could be something we integrate with new ticketing/queueing software.

Stage #4:

At the collection counter



Behavioural patterns

- **Identity verification is routine and familiar:** Customers are typically asked for a surname, medical aid details, or loyalty card. The process feels standardised and expected, especially at familiar branches.
- **Speed and predictability define success:** A smooth handover reinforces the value of pre-packed collection. Customers expect staff to quickly locate their medication without further explanation or delay.
- **Familiar staff relationships reduce effort:** At regularly used branches, staff recognition and rapport contribute to trust and a sense of ease during handover.

What participants said

“They just ask for my surname and then they get it.

— Collection participant

“They know us quite well... we have a really good relationship.”

— Collection participant



What worked well

- ✓ **Predictable handover steps:** Knowing exactly what will be asked for (name, card, details) reduces uncertainty.
- ✓ **Fast retrieval reinforces the ‘pre-packed’ promise:** Medication being ready behind the counter validates the effort of submitting scripts in advance.
- ✓ **Positive staff relationships build trust:** Familiarity with staff makes the process feel smoother and more personal.

Frictions

- ✗ **Variability across branches:** Differences in process or questioning can disrupt expectations.
- ✗ **Delays undermine confidence:** When medication is not immediately found, trust in the system is shaken.

Next steps & recommendations



Continue investigation of QR scanning to streamline identification and retrieval

As QR-based collection is tested, use it to reduce manual lookups, speed up handover, and minimise back-and-forth at the counter benefiting both pharmacists and customers.

Stage #5:

Counselling /information



Behavioural patterns

- **Routine reduces the need for counselling:** For repeat and chronic medication, most participants rely on prior knowledge and labels rather than expecting verbal explanations each visit.
- **Counselling is valued most for new or high-risk medication:** When medication is new, changed, or perceived as risky, customers actively want explanation and reassurance from pharmacy staff.
- **Labels act as the primary source of truth:** Printed labels are widely trusted for dosage and timing, especially when customers are already familiar with the medication.
- **Consistency builds confidence:** Familiar staff and repeated explanations over time reduce anxiety and make customers feel confident using their medication correctly.

What participants said

“I don’t really need them to explain every time — I know it already.”

— Collection participant

“If it’s something new, then I’ll ask.”

— Collection participant

“The label tells you what to do.”

— Collection participant



What worked well



Confidence built through routines:

Familiarity with taking the same medication over time reduces the need for repeated explanations and supports confident use.



Clear labelling supports self

management: *Labels that include dosage and usage instructions are sufficient for most participants.*



Counselling when needed is valued: *When explanations are provided especially for new medication they are seen as helpful and reassuring.*

Similar to previous research, there were no clear customer frictions at the counselling step.

Next steps & recommendations



Continue providing counselling based on

need and risk: *Maintain the current approach where pharmacists provide verbal counselling when medication is new, changed, or high-risk, while keeping repeat collections lightweight.*



Consistent, clear labelling *remains critical as the primary reference once customers leave the store.*



Future research (SOTF): *Investigate alternative ways for customers to ask medication-related questions outside of the collection counter such as post-collection WhatsApp support, follow-up messaging, or after-hours guidance.*

Stage #6:

Payment & leaving the store



Behaviourial patterns

- **Payment happens outside the pharmacy:** After collecting medication, customers are required to move to the front-of-store tills to complete payment.
- **Collection and shopping are often combined:** Many customers browse or shop for toiletries and specials either before or after collecting medication, especially when queues feel manageable.
- **Payment queues are the biggest source of frustration:** Long cashier queues, particularly during peak or month-end periods, undermine the time-saving benefit of pre-packed collection.
- **Cost coverage shapes emotional response:** When medication is fully covered by medical aid, the experience feels smooth; co-payments or mixed payments add friction and mental load.

What participants said

“Sometimes I just want to pay and go.”

— Collection participant

“Now you’ve got people that have done their monthly shopping... and you’ve got to pay for your meds.”

— Collection participant



What worked well



Coverage reduces decision stress:

Knowing that medication is largely covered by medical aid simplifies the payment moment for chronic users.



Opportunity to bundle errands: For some, combining payment with other shopping makes the trip feel more efficient overall.

What caused friction



Checkout queues negate time savings:

Fast collection followed by slow payment breaks the perception of efficiency.



Mismatch between task and effort:

Customers collecting only medication feel frustrated standing in long queues with full-trolley shoppers.



Lack of payment flexibility: Customers cannot choose whether to pay immediately or combine payment with other shopping.

Next steps & recommendations



The recommendation is similar to previous research which is to introduce **a point of sale at the pharmacy**. Alternatively, they could introduce a **medication only queue** at the front-of-shop tills.

These are both being investigated by Tribe 4 as part of the SOTF initiative.



Key recommendations across the collection journey

Focus area	Recommendation
Script handoff & confirmation	Introduce a clear, standard confirmation flow across all submission channels (in-store, email, WhatsApp, doctor-sent).
Digital trust & communication	Standardise and centralise email and Business WhatsApp handling so customers receive predictable responses regardless of branch. Reduce dependence on informal, staff-specific workarounds.
Notification & readiness signals	Make status updates explicit and differentiated (not generic “processing” messages). Align notifications to monthly repeat cycles and high-risk medication schedules rather than one-off alerts.
Identity & profile lookup (counter handover)	Accelerate counter handovers by rolling out QR-code scanning (WIP) , reducing repeated verbal checks and improving speed for both staff and customers.
Payment flow (end of journey)	Explore optional payment at or near the pharmacy counter for script-only purchases, reducing friction caused by front-of-store queues.
Counselling & ongoing support	Maintain situational counselling at collection , supported by clear labelling. Explore post-collection support channels (e.g. WhatsApp or digital pharmacist access) for questions that arise later.



A delivery person wearing a blue and yellow helmet and uniform is shown in profile, carrying a large cardboard box. The background is a blurred city street at night with bokeh lights. The text is overlaid on the right side of the image.

Insights and recommendations:

2. delivery script fulfilment journey

IN-DEPTH INTERVIEWS



SCRIPT JOURNEY OVERVIEW: DELIVERY

Stage 1: script submission / intake

How customers receive prescriptions and set up medication delivery at Dis-Chem. The focus is on **safely handing over** the original script (often in-store).

Stage 2: notification & planning for delivery

How customers are notified that medication is being processed or scheduled for delivery, and how they mentally plan around it.

Stage 3: waiting & tracking

This stage covers what happens between **notification and delivery arrival**. Customers typically wait at home **without live or precise tracking**, relying on general delivery windows or status messages.

Stage 4: delivery arrival & handover

Covers the courier arrival and handover of medication at the customer's home or workplace. Identity is confirmed verbally, medication is handed over quickly.

Stage 5: receiving & using medication

This stage covers the moment customers receive their medication, check that it is correct, and know how to use it.



Stage #1:

Script submission /intake



Behavioural patterns

- **Physical scripts remain the primary entry point:** Most participants still receive handwritten or printed prescriptions from doctors and bring them into a Dis-Chem branch to be captured.
- **Delivery is treated as a one-time setup, not a repeated choice:** Most participants describe delivery as something they activate once (often tied to a six-month chronic script), after which they expect medication to be delivered automatically on a monthly basis.
- **Responsibility is handed over after setup:** Once the script is captured and delivery is requested, they expect the pharmacy to manage the process end-to-end. Ongoing follow-up is not seen as part of the normal flow.
- **Branch staff play a key enabling role:** Pharmacy staff are central in explaining delivery, capturing scripts correctly, updating address details, and confirming that delivery has been set up.

What participants said

"I took the script and told them to deliver it."

— Delivery participant

On setting up delivery: "I normally just drop it (script) off."

— Delivery participant



What worked well

- ✓ **Clear activation moment:** *Handing in the physical script and explicitly requesting delivery gives customers confidence that the process has been set up correctly.*
- ✓ **Low ongoing effort once delivery is active:** *Once delivery is running, customers expect medication to arrive without repeated decisions, coordination, or reminders.*
- ✓ **Human support at setup:** *In-branch assistance reduces anxiety during initial setup, particularly for chronic medication and first-time delivery users.*

What caused friction

- ✗ **Low trust in “invisible” handoff:** *App, email, or WhatsApp submission lacks tangible confirmation that the script has been correctly loaded for delivery.*
- ✗ **Initial setup is perceived as higher-risk than ongoing delivery,** *which explains why customers prefer physical handover at this stage.*
- ✗ **Customers self-manage risk through presence:** *Physical drop-off is used as a risk-mitigation strategy, not because digital channels are unavailable.*



Next steps & recommendations



Design digital setup to feel as safe as in-store handover: Explicit confirmation that delivery is active, repeats are loaded, and addresses are correct not just “script received.”



Acknowledge setup vs. repeat as different moments: Treat first-time delivery setup as a high-trust moment with stronger reassurance than ongoing monthly repeats.



Make physical and digital paths equally valid: Support in-store setup without friction, while gradually building trust in digital setup through clearer proof and status feedback.



Stage #2:

*Notification & planning for
delivery*



Behavioural patterns

- **Notifications are used as reassurance, not planning tools:** Most participants interpret messages as confirmation that their medication is “being handled,” rather than as information to plan their day around.
- **Delivery is expected to progress automatically once set up:** Once a script is loaded, customers expect delivery to follow without additional action.
- **Timing clarity varies and expectations are adjusted accordingly:** Some participants receive clearer delivery windows, while others expect only approximate timing and adjust their behaviour to “wait and see.”

What participants said

"It's more just to know that things are moving."

— Delivery participant

"They let you know it's being processed."

— Delivery participant



What worked well



Low effort, low decision-making:

Customers don't need to actively plan or confirm next steps once notified.



Reassurance through communication:

Even high-level notifications reduce anxiety by confirming progress.

What caused friction



Unclear distinction between statuses:

"Processing," "on the way," and "delivering" can blur together, reducing usefulness for expectation-setting.



Customers adapt expectations

downward: *Rather than demanding clarity, participants lower expectations and prepare to wait.*

Next steps & recommendations



Clarify delivery status states: Clearly differentiate between script loaded, delivery scheduled, and out for delivery (delivery windows)



Use notifications to manage confidence, not just inform: Reinforce that no further action is required once delivery is confirmed.



Stage #3:

Waiting & tracking



Behavioural patterns

- **Delivery is chosen by people who can afford to wait:** Participants who use delivery tend to have work-from-home flexibility, shift patterns, or someone available at home. Waiting is therefore acceptable, even if timing is vague.
- **Approximate delivery windows are expected:** Participants understand that delivery timing varies by store and generally expect a broad window (e.g. a few hours), rather than precise or live tracking.
- **Waiting is passive and trust-based:** Customers do not actively track deliveries. They rely on the assumption that medication will arrive once notified.
- **Uncertainty is tolerated, but noticed:** delivery is chosen specifically by customers whose work or household setup allows them to wait. While not described as a major problem, not knowing when delivery will arrive creates low-level discomfort and limits planning.

What participants said

“You’re not always sure when they’re coming.”

— Delivery participant

“You just wait at home.”

— Delivery participant



What worked well

- ✓ **Delivery fits existing lifestyles:** Participants choose delivery precisely because being at home is already feasible.
- ✓ **Trust outweighs the need for tracking:** Customers trust that medication will arrive and do not feel the need to monitor progress closely.
- ✓ **Low effort once delivery is set up:** No follow-ups or active management are required during the waiting phase.

What caused friction

- ✗ **Limited ability to plan the day:** Broad delivery windows make it difficult to schedule errands, meetings, or outings.
- ✗ **Uncertainty increases background stress:** Even when tolerated, not knowing timing adds low-level tension.
- ✗ **Experience varies by store:** Delivery windows and communication consistency depend on branch-specific practices.



Next steps & recommendations



Align future improvements with operational changes: As new delivery SLAs, app updates, and protocols are introduced, ensure communication explicitly supports expectation-setting rather than real-time tracking.

Stage #4:

Delivery arrival & handover



Behavioural patterns

- **Handover is fast and low-interaction:** Participants consistently describe delivery handover as quick, minimal, and efficient, with no expectation of conversation or explanation.
- **Identity confirmation is lightweight:** Verification typically involves confirming a name verbally. Formal ID checks are not common and are not expected by customers.
- **Medication arrives sealed and discreet:** Packaging is consistently described as sealed and non-transparent, reinforcing trust and privacy.

What participants said

“They just check your name and give it to you.”

— Delivery participant

“It’s wrapped up and sealed well... he can hand it to me at the gate.”

— Delivery Participant



What worked well

- ✓ **Quick, predictable handover:** Delivery interactions are consistently short and efficient, matching expectations for a routine service.
- ✓ **Clear privacy signals:** Sealed, discreet packaging reassures customers that medication is handled appropriately.
- ✓ **No cognitive or emotional effort required:** Customers do not need to explain, verify details repeatedly, or engage beyond confirming identity.

No significant frictions reported at handover

Participants did not raise concerns about the delivery interaction itself, suggesting this stage is already well-aligned with customer expectations.

Next steps & recommendations

- 💡 **Design medication support outside the fulfilment moment:** Similar to collection customers, who only want counselling when needed, delivery customers do not expect or want discussion at handover. Any additional support should happen around the journey, not during it.
- 💡 **Test shared post-handover support models across collection and delivery:** Explore a single, consistent support layer that works for both journeys, such as:
 - Optional WhatsApp or app-based pharmacist follow-up
 - Telepharmacy or call-back support for questions after receiving medication
 - Clear post-handover prompts (“Need help with your meds?”)



Stage #5:

Receiving & using medication



Behavioural patterns

- **Trust is established through packaging and consistency:** Sealed, discreet packaging signals that medication is safe and correct, reducing the need for detailed checking.
- **Accuracy is assumed, not verified:** Customers rarely inspect contents closely. Confidence is built through repeated reliability rather than active confirmation.
- **Labels replace conversation:** Printed labels are the primary source of information. Customers rely on them, and prior knowledge, rather than expecting counselling.
- **Routine overrides attention:** For chronic medication, familiarity means customers often don't re-read instructions unless something changes.

What participants said

"You can't see that it's medication from the outside."

— Delivery participant

"I haven't properly looked at it... we're so used to it."

— Delivery participant



What worked well

- ✓ **Strong trust signals at receipt:** Packaging and consistency provide reassurance without effort.
- ✓ **Low cognitive load:** Customers do not need to follow up, check, or seek clarification.
- ✓ **Supports long-term routines:** Delivery integrates smoothly into chronic medication habits.

What caused friction

- ✗ **No immediate support if uncertainty arises:** While not reported as a problem, delivery offers no built-in way to ask questions if something feels unclear. Customers would have to call into the store.



Next steps & recommendations

The recommendations are the same as stage 4:



Design medication support outside the fulfilment moment: *Similar to collection customers, who only want counselling when needed, delivery customers do not expect or want discussion at handover. Any additional support should happen around the journey, not within it.*



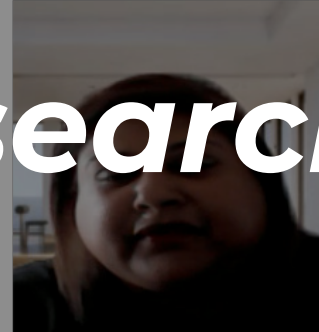
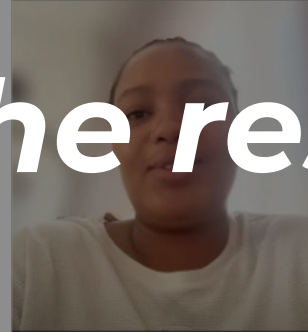
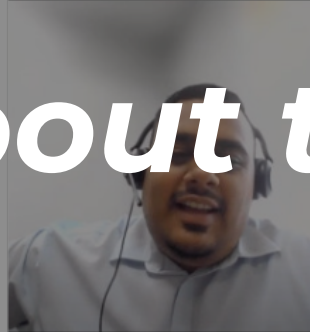
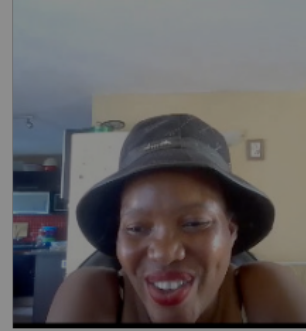
Test shared post-handover support models across collection and delivery: *Explore a single, consistent support layer that customers can access if needed, including:*

- *Optional WhatsApp or app-based pharmacist follow-up*
- *Telepharmacy or call-back support for questions after receiving medication*
- *Clear post-handover prompts (“Need help with your meds?”)*

Key recommendations across the delivery journey

Focus area	Recommendation
Delivery setup & script intake	Make the digital setup path feel as reassuring as in-store handover : provide clear proof of receipt, visible status (“received / captured / scheduled”), and an easy way to confirm details so customers don’t default to physical drop-off for certainty.
Consistency across branches	Standardise delivery setup and expectations across branches so customers experience the same process regardless of where they initiate delivery.
Notification & delivery windows	Set clear expectations around delivery timing (e.g. standard 2–3 hour windows) and ensure notifications communicate what will happen next, not just that processing is underway.
Waiting & tracking expectations	Maintain a low-effort waiting experience for customers who choose delivery because they can be at home, while improving clarity rather than introducing complex live tracking.
Handover & verification	Preserve the current fast, low-interaction handover model , which customers perceive as efficient and appropriate for medication delivery.
Post-delivery support	Design optional, post-handover support (shared with Collection), such as WhatsApp pharmacist support or call-back options, rather than introducing counselling during delivery.





About the research



Research plan and process

- 15 - 19 December 2025
- Remote in-depth interviews with 15 Dis-Chem customers (8 collection customers, 7 delivery customers)
- Set up a screener and brief the recruiter
- Create and socialise a research protocol
- Ensure all participants sign consent & non-disclosure agreement forms
- Each time slot should have:
 - 1 facilitator
 - 1 observer, that also serves as note taker
- $\pm$ 1 hour for each session = 15 hours
- Complete note taking and synthesise data
- Compile a research report with insights and recommendations



Research method

In-depth interviews is a **qualitative** methodology that helps us understand the **lived experience, sentiment** and **behaviour** and allows us to create follow-up research studies informed by customers, not our own pre-conceived ideas.

We engage with a research recruitment company to find representative customers by setting up a participant screener.

Based on the insights from the first set of qualitative research, we will design more qualitative studies if needed, or move into quantitative methods like surveys, if we feel confident, we know what survey questions will be most useful to the business.





ABOUT THE RESEARCH

Research team



***Camilla
Cicatello -***
Researcher



Dis-Chem PHARMACY

Thanks

